HMI Client Contact Hours (CCH) Log

Student: Jithendran Sellamuthu
AHA #: 007913
Client Initials: T.M.
Session #: 2a
Date: October 16, 2026
Duration: 90 minutes
Modality: Zoom (Virtual)
Fee: Pro Bono (CSP)

1. Presenting Issue

T.M. is a 22-year-old student at Appalachian State University in North Carolina. He was referred through the HMI Community Service Program (CSP). He presents with anxiety, hypervigilance, and trust difficulties stemming from childhood trauma involving abuse by family members including his mother. Core wounds include abandonment, betrayal, and shame. Specific triggers: men yelling (fear/anxiety response), feeling disrespected or demeaned, and vulnerability in group settings. Client has completed traditional psychotherapy which provided significant cognitive insight but seeks hypnotherapy to address subconscious patterns that talk therapy has not fully resolved. Suggestibility profile established in Session 1: 31% Physical / 69% Emotional.

2. What preparation did you do for this session?

- Reviewed Session 1 intake notes, suggestibility result (69% Emotional), and SOAP notes
- Prepared a 12-step session script following the Lisa Machenberg wound-identification model adapted for Emotional delivery lane
- Prepared three primary wound descriptions (abandonment, betrayal, shame) for client identification
- Prepared formal Emotional-lane induction script (inferential arm-raising, 5-0 count, staircase, progressive relaxation)
- Prepared safe space construction protocol, desensitization script (adult views younger self), and wound-specific hypnotic intentions
- Prepared EFT reinforcement protocol with wound-specific setup statements
- Reviewed HMI case conference guidance (John Melton on inner child, George Kappas on reprogramming, Saman Nasir on neuroscience, Lisa Machenberg on wound identification and desensitization)
- Prepared SMART baseline collection protocol (6 scales, 1-10) deferred from Session 1

3. What hypnotic techniques and suggestions did you use in this session?

Session adapted from the prepared wound-identification protocol to address a triggering incident the client brought from his workplace. This adaptation was clinically appropriate and followed the client's presenting material. The following techniques were employed:

- **Identity vs. Behavior Separation:** Established the framework that the client is not his reactions. Emotions are contextual, triggered by specific inputs. Self-labeling (calling himself "asshole" when judgmental) binds behavior to identity and damages self-esteem. Used analogy of roles in life (son, boyfriend, employee) and the Hindu lotus leaf metaphor (living amid difficulty without being defined by it).
- **Jungian Projection / Shadow Work Concepts:** Discussed how judgments of others often reflect internal wounds and fears. Reframed the client's judgment of the park customer as a defensive emotional counterattack rather than a character flaw.
- **Desensitization Framework:** Explained desensitization as reducing how much a trigger affects the ego and self-esteem, distinguished from being "numb." Used the police officer analogy (following protocol calmly regardless of emotion) and the Snake and Saint parable (defend but do not attack).
- **Induction:** Breathing lead-in (3 cycles) followed by spoken affirmations with hand on chest ("I am great," "I am a wonderful person," "Everybody genuinely likes me," "I am being loved"). Eyes-open visual anchoring (client focused on a spot on the wall and visualized his own face expressing pride, bravery, and fulfillment). Eye closure (permissive). "Deep Sleep" keyword established as the hypnotic induction phrase with client's permission.
- **Deepening:** 5-0 countdown followed by 10-0 staircase, progressively deepening the trance state. Full progressive body scan from crown to toes with therapeutic reframe: negative life experiences reframed as purposeful learning that built empathy so the client could understand others' pain and help them.
- **Hypnodrama — Tribe Circle:** Client visualized a safe room with 11 people seated in a circle — his "soul tribe," each radiating a specific quality (bravery, motherly care, self-esteem, belonging, and others). He was guided to invite each person into the circle one by one, feeling the energy shift as the group grew. Each tribe member was associated with a distinct supportive energy. All 11 were then integrated into the client's body sequentially, sealed with a golden light forming a protective aura. This directly addressed the client's fear of vulnerability in group settings.
- **Anchor Installation:** Physical anchor installed: pressing thumb and forefinger together on the right hand invokes the integrated, protected, self-assured state built during the tribe circle visualization. Reinforced twice during the session to deepen the association.
- **Rescuer-Helper Identity Reframe:** Synthesized two positive memories (childhood beach vacation where client felt safe and content; river-tubing rescue where client freed a friend's trapped wrist and felt genuine pride) into a "rescuer-helper" archetype. Reframed others' anger as a call for help, past wounds as empathy training, and the therapeutic arc as a transition from victim mindset to healer-helper identity.

- **Count Out:** Guided return via staircase (steps 1-10), each step associated with growing confidence, dignity, and pride. Subconscious "door" symbolically closed. Counted out from 1 to 5 with reframe: "Everything that happened was an experience so you can help others empathetically. You are beyond being judged."

4. What were the highlights of this session?

- **S1-to-S2a progress:** Client reported that since Session 1, it became "easier to observe thoughts and emotions without immediately reacting — watching them arise and letting them be, even when negative." This spontaneous observer-stance is a significant developmental marker and indicates strong between-session processing.
- **Self-labeling identified:** Client disclosed that he calls himself "asshole" or "stop being so judgmental" when he has negative thoughts about others. This self-labeling pattern (behavior = identity) was identified as a primary target for therapeutic intervention.
- **Triggering incident provided rich clinical data:** The workplace scenario (park customer) revealed the client's core behavioral loop: perceived disrespect triggers defensive anger, followed by guilt about the anger, followed by self-labeling as a "bad person," followed by shame. This cycle is the specific pattern to interrupt.
- **Additional clinical material disclosed:** Recurring nightmares (fighting, being attacked, revenge), guilt about withdrawing from a female friendship with romantic undercurrent, and explicit fear of vulnerability in therapy group settings (fears judgment, being made to feel "less than").
- **Hypnodrama tribe circle landed effectively:** Client engaged well with the 11-member visualization. The sequential integration and golden light seal created a tangible protective experience. Post-trance, client reported feeling "more steady" and described the sensation as "almost like an armor or a shield" — indicating the integration exercise and anchor installation were successful.
- **Session adapted appropriately:** The prepared wound-identification protocol was set aside in favor of following the client's presenting material (workplace trigger, self-labeling pattern, group vulnerability fear). The hypnodrama tribe circle addressed the same therapeutic targets (trust, vulnerability, self-worth) through an experiential route rather than the scripted cognitive route. This is consistent with the Emotional lane — imagery and experience over analysis.

5. Did you discuss this session with a supervisor and if so what guidance did you receive?

Prior to this session, I reviewed relevant case conference guidance from three HMI instructors:

- **Lisa Machenberg:** Three primary wounds model (abandonment, betrayal, shame). Target emotions should be positive ("how do you wish you felt?"), no negatives. Anchor with positive memory. Wound-specific hypnotic intentions. Same trigger, new behavior, superior result. The wound identification was addressed organically through the triggering incident rather than the formal 3-wound presentation.
- **John Melton:** Check expectations. Inner child work: adult self re-parents (from current perspective, not regression). Room must be NEW (not from childhood). Watch for sabotaging behavior. Don't vary induction just for variety — repetition = association. Applied: kept induction consistent with established affirmation style, client-designed visualization.
- **George Kappas:** Don't dwell in the past — address the PROGRAMMING, not the story. Therapy-experienced clients may intellectualize instead of experiencing. Applied: moved quickly from analysis to experiential trance work. Client engaged with the experience rather than analyzing it.

6. What do you plan to do for the next session?

- **Collect SMART baselines:** Six self-report scales (1-10) deferred from Sessions 1 and 2a. Non-negotiable for Session 2b.
- **Circle Therapy (Kappasinian fear extinction):** Systematic desensitization using the principle that anxiety and deep relaxation are physiologically incompatible. Four confrontation cycles in trance: (1) disrespect/demeaning at low intensity, (2) disrespect at medium intensity with rescuer-helper reframe, (3) vulnerability in group settings (therapy circle scenario), (4) trusting someone close (gated, optional based on readiness).
- **Reversal suggestion:** Client attempts to summon the fear deliberately — discovers it cannot sustain itself after extinction cycles.
- **Compound the anchor:** Link the existing thumb-forefinger anchor (tribe circle state) with the protective circle from Circle Therapy, creating a multi-layered anchor.
- **Monitor nightmare pattern:** If persisting, may indicate incomplete processing requiring additional sessions.
- **Future sessions:** Circle of Trust (graduated trust-building visualization) planned for S3. Guilt/friendship processing (cord cutting or hypnodrama) for S4.